

1) Patient Report

Encounter Type: Inpatient Admission

Date of Encounter: [Insert Date]

Patient Name: [Insert Patient Name]

Medical Record Number: [Insert MRN]

Age: 40 years

Gender: [Insert Gender]

Chief Complaint:

Fatigue, Weakness and Shortness of breath on exertion.

History of Present Illness:

The patient is a 40-year-old individual with a known history of **advanced colon cancer**, who presents with worsening symptoms of anemia, including fatigue and exertional dyspnea. Laboratory tests revealed significant reduction in hemoglobin and hematocrit levels. The anemia is attributed to the underlying malignancy. The patient is admitted for management and treatment of anemia, including transfusion support and evaluation for iron supplementation and/or erythropoiesis-stimulating agents.

Assessment and Plan:

1. **Anemia due to malignancy (colon cancer)**
2. **Advanced colon cancer (primary site: colon)**

The patient will receive blood transfusions and undergo further evaluation by oncology and hematology teams to manage the anemia and review ongoing cancer treatment.

ICD-10-CM Diagnosis Codes:

Diagnosis Description	ICD-10-CM Code
Anemia in neoplastic disease	D63.0
Malignant neoplasm of colon (e.g., unspecified site)	C18.9

Coding Note:

- **D63.0** (Anemia in neoplastic disease) should **not be used as the principal diagnosis**.
- The principal diagnosis should be **C18.9** (Colon cancer), because the admission is for management of a condition (anemia) related to the cancer.

2) Patient Report (14)

Encounter Type: Outpatient Evaluation

Date of Encounter: [Insert Date]

Patient Name: [Insert Patient Name]

Medical Record Number: [Insert MRN]

Age: 30 years

Gender: Female

Gestational Age: Second Trimester (approximately 14–27 weeks)

Chief Complaint:

Neck mass and concern for thyroid malignancy during pregnancy.

History of Present Illness:

A 30-year-old pregnant female in her second trimester presents for evaluation of a thyroid mass that was recently discovered during a routine prenatal examination. Ultrasound imaging and fine-needle aspiration biopsy suggest a diagnosis of **thyroid cancer**. The patient has no prior history of thyroid disease. She is currently in the second trimester of pregnancy, and the malignancy is impacting her clinical management.

Assessment and Plan:

1. **Malignant neoplasm of the thyroid gland**
2. **Pregnancy complicated by a neoplasm (thyroid cancer)**
3. **Pregnancy, second trimester**

The patient will be referred to endocrine oncology and maternal-fetal medicine to discuss management options. Multidisciplinary care will be coordinated due to the pregnancy and malignancy.

ICD-10-CM Diagnosis Codes:

Diagnosis Description	ICD-10-CM Code
Malignant neoplasm of thyroid gland	C73
Pregnancy complicated by neoplasm, second trimester	O9A.112
Weeks of gestation not specified in trimester documentation (if needed)	Z3A.00

Coding Notes:

- **O9A.112** is used when pregnancy is complicated by a malignant neoplasm in the **second trimester**.
- **C73** specifies **malignant neoplasm of the thyroid gland**.
- **Z3A.00** can be used to indicate **weeks of gestation not specified**, but a more precise gestational age (e.g., **Z3A.18** for 18 weeks) is preferred if known.

3) Patient Report

Encounter Type: Inpatient Postoperative Management

Date of Encounter: [Insert Date]

Patient Name: [Insert Patient Name]

Medical Record Number: [Insert MRN]

Age: [Insert Age]

Gender: [Insert Gender]

Chief Complaint:

Postoperative glucose management in a patient with Type 2 diabetes.

History of Present Illness:

The patient is a known case of **Type 2 diabetes mellitus**, typically well-controlled on **oral metformin therapy**. During this inpatient admission for [insert type of surgery], glucose levels fluctuated due to surgical stress and recovery. The patient was temporarily placed on **short-term insulin therapy for 3 days** to maintain glycemic control while off regular oral medications.

The patient had no evidence of diabetic complications, and insulin was discontinued once oral intake and metabolic status stabilized. The diabetes remains without complications.

Assessment and Plan:

1. **Type 2 diabetes mellitus without complications**
2. **Temporary insulin use for glycemic control during hospitalization**

The patient will resume metformin once fully recovered and discharged. Endocrinology follow-up is advised to reassess outpatient diabetes management.

ICD-10-CM Diagnosis Codes:

Diagnosis Description	ICD-10-CM Code
Type 2 diabetes mellitus without complications	E11.9
Encounter for short-term insulin use (optional/clinical context)	Z79.4 (<i>Long-term use only – typically not used for short-term inpatient insulin</i>)

Note:

- **E11.9** is appropriate as the **primary diagnosis** to represent Type 2 diabetes without complications.
- **Z79.4** ("Long-term [current] use of insulin") **should not be coded** unless the patient is expected to **continue insulin therapy indefinitely** after discharge. Since insulin was used **only temporarily during hospitalization**, **Z79.4 should not be assigned** in this case.

4) Patient Report

Patient Name: [Insert Patient Name]

Date of Admission: [Insert Date]

Age/Sex: [Insert Age and Gender]

Chief Complaint: Ongoing cannabis use

History of Present Illness:

The patient has a documented history of chronic cannabis use. According to the History and Physical (H&P) note, the patient meets the criteria for **cannabis dependence**, characterized by continued use despite adverse consequences, tolerance, and inability to reduce usage. The progress note, however, refers to the condition as **cannabis abuse**, suggesting episodic use with social or occupational impairment but not meeting full dependence criteria.

Assessment:

Given the more severe diagnosis in the H&P and current clinical assessment, the patient is best

categorized as having **cannabis dependence**. DSM-5 has replaced the separate categories of abuse and dependence with **Cannabis Use Disorder**, which encompasses a spectrum of severity.

Plan:

- Further evaluation by psychiatry or addiction specialist
 - Counseling and behavioral therapy referral
 - Education on substance use effects
 - Monitor for withdrawal symptoms
 - Follow-up appointment scheduled
-

ICD-10 Code

- **F12.20** — Cannabis dependence, uncomplicated
(This is used when cannabis use meets criteria for dependence without intoxication or other complications.)

Note: If the documentation supports only "abuse" without criteria for dependence, the code would be **F12.10** — Cannabis abuse, uncomplicated.

However, since H&P notes typically carry more diagnostic weight, **F12.20** is more appropriate unless clarified otherwise.

5) Patient Report

Patient Name: [Insert Patient Name]

Date of Admission: [Insert Date]

Age/Sex: [Insert Age and Gender]

Chief Complaint: Abdominal distension and fatigue

History of Present Illness:

The patient presents with signs and symptoms consistent with advanced liver disease, including fatigue, abdominal distension, and jaundice. The patient has a known history of chronic alcohol use and meets the criteria for alcohol dependence. The current clinical picture and imaging/laboratory findings confirm the diagnosis of **alcoholic cirrhosis of the liver**.

Assessment:

- Alcoholic cirrhosis of liver
- Alcohol dependence, uncomplicated

Plan:

- Initiate supportive treatment for liver disease
 - Monitor liver function tests
 - Nutritional support and vitamin supplementation (especially thiamine and folate)
 - Refer to addiction services for alcohol cessation support
 - Consider hepatology consult
 - Educate on alcohol cessation and long-term liver management
 - Arrange follow-up in liver clinic
-

ICD-10 Codes

- **K70.30** — Alcoholic cirrhosis of liver without ascites
(Use **K70.31** if ascites is present)
- **F10.20** — Alcohol dependence, uncomplicated

If the patient also experiences withdrawal, intoxication, or other complications, a more specific code under **F10.2x** should be selected.

6) Patient Report

Patient Name: [Full Name]

Date of Examination: [MM/DD/YYYY]

Medical Record Number: [XXXXXX]

Physician: [Physician's Name]

Department: Neurology

Subjective:

The patient reports weakness and loss of movement on the left side of the body. Symptoms developed [onset date or duration, if known], and there is no reported trauma. The patient identifies as ambidextrous, with equal use of both hands prior to the onset of symptoms.

Objective:

Neurological examination reveals flaccid paralysis and muscle weakness on the left side of the body. Reflexes are diminished on the affected side. No movement is noted in the left upper and lower extremities. Cranial nerves are intact. No evidence of head trauma. Initial CT and MRI imaging pending.

Assessment:

Hemiplegia affecting the left side in an ambidextrous individual.

Plan:

- Admit for further evaluation and imaging (CT/MRI brain).
 - Neurology consultation for further diagnostic workup.
 - Begin physical and occupational therapy as tolerated.
 - Monitor for progression of symptoms.
 - Rule out underlying etiology (e.g., stroke, tumor, demyelinating disease).
-

★ Diagnosis Code (ICD-10):

G81.04 – *Hemiplegia, unspecified affecting left nondominant side*

Note: For ambidextrous patients, the **nondominant side** is used by coding convention unless a different side is clearly more impaired or historically dominant.

7) Patient Report

Chief Complaint:

Bilateral upper eyelid swelling.

Clinical Findings:

The patient presents with symmetrical swelling (edema) of both the right and left upper eyelids. The affected areas exhibit mild tenderness upon palpation. There is no evidence of erythema, discharge, or recent trauma. Ocular movements and visual acuity are within normal limits. No signs of systemic illness or allergic reaction are currently observed.

Assessment/Diagnosis:

- Edema of right upper eyelid (ICD-10-CM Code: H02.841)
- Edema of left upper eyelid (ICD-10-CM Code: H02.844)

Plan:

Further evaluation to determine the underlying cause of the eyelid edema is recommended. Consider allergy testing, complete blood count (CBC), and assessment for possible systemic conditions (e.g., thyroid dysfunction, renal issues). Symptomatic treatment may include cold compresses and monitoring for any progression.

8) Patient Report

Patient Information:

Age: 40 years

Gender: Female

Chief Complaint:

Bilateral eye pain.

History of Present Illness:

The patient, a 40-year-old woman, presents with complaints of pain in both eyes. She reports discomfort localized to the upper eyelids, described as a persistent, irritating sensation.

Clinical Findings:

On physical examination, there is evidence of inflammation along the margins of the upper eyelids bilaterally. Findings are consistent with blepharitis, including redness, irritation, and crusting at the base of the eyelashes. No significant discharge or signs of acute infection are present.

Diagnosis:

Bilateral blepharitis of the upper eyelids, ICD-10-CM Code: H01.001, H01.004

Plan:

- Initiate eyelid hygiene regimen (warm compresses, lid scrubs)
- Consider topical antibiotic or anti-inflammatory treatment if symptoms persist
- Schedule follow-up in 2 weeks to monitor progress
- Rule out underlying contributing factors (e.g., seborrheic dermatitis, rosacea)

9) Patient Report

Patient Information:

Age: 55 years

Gender: [Insert Gender]

Date of Examination: [Insert Date]

Chief Complaint:

Skin breakdown and pain in the lower back area.

History of Present Illness:

The patient, a 55-year-old individual, presents with a pressure ulcer located on the right lower back. The ulcer is covered by thick, adherent eschar, making it impossible to clinically determine the depth of tissue damage at this time. The area is non-draining, and there are no signs of acute infection, but the wound is considered serious due to its unstageable status.

Clinical Findings:

- Pressure ulcer on the right lower back
- Wound base obscured by necrotic tissue (eschar)
- No visible exposure of bone, tendon, or muscle
- No purulent discharge or foul odor at this time

Diagnosis:

Unstageable pressure ulcer of the right lower back .

ICD-10-CM Code: L89.310 – *Pressure ulcer of right lower back, unstageable*

Plan:

- Initiate appropriate wound care (e.g., debridement if indicated)
- Monitor for signs of infection
- Pressure offloading and repositioning protocol
- Nutritional assessment and support
- Wound care team consultation

10) Patient Medical Record

Patient ID: [Insert Patient ID]

Date: [Insert Date]

Name: [Insert Patient Name]

Age/Gender: [Insert Age/Gender]

Chief Diagnosis:

- Type 1 Diabetes Mellitus
- Chronic Kidney Disease (CKD), Stage 2

Medical History:

- Known case of Type 1 Diabetes Mellitus, on insulin therapy
- Diagnosed with Stage 2 Chronic Kidney Disease (eGFR 60–89 mL/min/1.73 m²)
- No history of dialysis or renal replacement therapy

Clinical Notes:

- Mild decrease in renal function, likely related to long-standing diabetes
- Monitoring of renal function and glycemic control ongoing
- No signs of acute kidney injury
- Urinalysis and renal imaging consistent with early diabetic nephropathy

Current Medications:

- [Insert Insulin regimen]
- [Other medications if applicable]

Plan:

- Continue insulin therapy and adjust based on blood glucose levels
- Monitor eGFR, serum creatinine, and urinary albumin every 3–6 months
- Maintain blood pressure within target range (e.g., <130/80 mmHg)
- Dietary and lifestyle counseling for renal protection
- Consider nephrology referral for baseline assessment

Diagnosis: Type 1 Diabetes Mellitus with Chronic Kidney Disease, Stage 2

- **ICD-10 Codes:**
 - E10.22 – Type 1 diabetes mellitus with diabetic chronic kidney disease
 - N18.2 – Chronic kidney disease, stage 2 (mild)

Clinical Summary:

The patient is a known case of **Type 1 Diabetes Mellitus**, currently presenting with **Chronic Kidney Disease (CKD) Stage 2**, indicating a mild reduction in kidney function (eGFR between 60–89 mL/min/1.73 m²). Kidney damage is likely secondary to long-standing diabetes (diabetic nephropathy).

Management Plan:

- Continue and monitor insulin therapy for glycemic control
- Regular renal function tests: eGFR, serum creatinine, and urine albumin-to-creatinine ratio

- Monitor blood pressure and manage as needed
- Dietary counseling with renal and diabetic considerations
- Consider referral to nephrology for early intervention
- Educate patient on signs of CKD progression

Follow-Up:

- Every 3–6 months or as clinically indicated based on lab results